



YMNOTES MEDICAL · COMPANION CAPSULE

YMnotes Digestive System

Other Subjects Capsule

Pharmacology, pathology, microbiology and parasitology points from **GIT Others** that your book does not already contain. They are arranged by your chapters, across both halves of the book.

19

additions

6

chapters with entries

6

points to check



HOW TO USE

About this capsule

A short teaching supplement to **YMnotes Digestive System • Version 3**. When you finish a chapter, open that chapter here. Each entry adds only what **GIT Others** contributes beyond your book: a fact, a distinction, a drug or laboratory detail, or a short mechanism. Points your book already makes, even in other words, are left out.

Page references

Book p. 196 is the **printed page number** at the foot of your book's page.

1st-half PDF p. 199 or **2nd-half PDF p. 135** is the page number your PDF viewer shows in that file. It is a different number, given only to help you find the page.

GIT Others p. 20 is the page in GIT others.pdf. Its PDF page number and printed page number are the same.

Subjects and checks

Each entry is labelled with the subject it comes from:

PHARMACOLOGY

PATHOLOGY

MICROBIOLOGY

PARASITOLOGY

CHECK with a dashed border marks a place where the sources disagree, or where your book disagrees with itself. Both versions are quoted and neither is chosen for you.

Your book's two files and where they join

File	Printed pages	Converting a viewer page	Chapters
1st half · 4_5875397832227168769.PDF (200 PDF pages)	1–197	printed = PDF page – 3	1–14, and the start of 15
2nd half · 4_5875397832227168769(1).PDF (238 PDF pages)	197–433	printed = PDF page + 196 (PDF p. 238 is the closing page, with no number)	the rest of 15, then 16–40

The files join **inside Chapter 15, Colorectal Cancer** (pp. 195–209). Printed page 197 is in both files. Chapter 15 is treated as one chapter here.

All 47 pages of GIT Others were compared with both files of YMnotes Digestive System, Version 3 (printed pp. 1–433). Every entry, and every point left out, is recorded in the comparison ledger that comes with this capsule.





CONTENTS

Contents

In your book's chapter order. Tap a row to go to it. The number on the right is the page in this capsule. The book pages are printed page numbers.

CHAPTERS WITH ADDITIONS

03	The Stomach and Duodenum Internal Medicine · Pharmacology · Surgery PHARMACOLOGY PATHOLOGY	Book pp. 33–69 · 1st-half file · 4 entries	4
04	Pathology of GIT Tumours Pathology · Surgery PATHOLOGY	Book pp. 70–75 · 1st-half file · 8 entries	4
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14	Polyposis of Colon Surgery · Pathology PATHOLOGY	Book pp. 189–194 · 1st-half file · 1 entry	7
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15	Colorectal Cancer Surgery · Pathology PATHOLOGY	Book pp. 195–209 · Spans both files · 4 entries	7
25	Pyogenic Liver Abscess Surgery · Internal Medicine · Parasitology PARASITOLOGY	Book pp. 324–331 · 2nd-half file · 1 entry	8
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03

The Stomach and Duodenum

Internal Medicine · Pharmacology · Surgery · Main book pp. 33–69 1st-half file

First-half file, PDF pp. 36–72. GIT Others pp. 1–12 (drugs for gastritis and peptic ulcer, with MCQs) repeat pp. 44–50 almost word for word.

Fits after: Pharmacology → 1. Proton Pump Inhibitors → Adverse effects · **Book p. 45** (1st half PDF p. 48)

Long-term PPI use and bone

Osteoporosis is a possible complication of chronic (long-term) PPI use. Add it to the PPI adverse effects, after vitamin B12 deficiency.

PHARMACOLOGY

GIT Others p. 11 (MCQ 24; answer key p. 12)
C3-1
ledger PH-26

Fits after: NSAID-induced ulcer → prophylaxis (misoprostol) · **Book p. 37** (1st half PDF p. 40) · also p. 48 (1st-half PDF p. 51)

Misoprostol: PGE1 or PGE2 analogue?

- ◆ Your book p. 37: “a synthetic prostaglandin **E₂** analogue”.
 - ◆ Your book p. 48 and GIT Others p. 4: “Synthetic **PGE1** analog”.
- Your two pages disagree. GIT Others agrees with p. 48.

PHARMACOLOGY

CHECK

GIT Others p. 4
C3-D1
ledger PH-15

Fits after: H. pylori eradication → rescue (quinolone) therapy · **Book p. 41** (1st half PDF p. 44) · also p. 48 (1st-half PDF p. 51)

Levofloxacin dose in the rescue regimen

- ◆ Your book p. 41: “Levofloxacin **500 mg once daily** + PPI + Amoxicillin”.
- ◆ Your book p. 48 and GIT Others p. 5: “levofloxacin **250 or 500 mg twice a day** for 10 days” (with PPI + amoxicillin 1 g twice a day).

Your two pages disagree. GIT Others agrees with p. 48.

PHARMACOLOGY

CHECK

GIT Others p. 5
C3-D2
ledger PH-16

Fits after: Cancer stomach → Site · **Book p. 62** (1st half PDF p. 65)

Commonest site of gastric cancer: how common?

- ◆ Your book p. 62: antrum/pyloric area **60%**, body 15%, fundus 25%.
- ◆ GIT Others p. 15: pyloric region **50%**, then **lesser curvature 25%**, but it can occur anywhere in the stomach.

Both sources agree the pyloric (antral) region is the commonest site. They give different percentages and a different second site.

PATHOLOGY

CHECK

GIT Others p. 15
C3-D3
ledger TU-13

04

Pathology of GIT Tumours

Pathology · Surgery · Main book pp. 70–75 1st-half file

First-half file, PDF pp. 73–78. Most of GIT Others pp. 13–21 is already on these pages.

Fits after: Carcinoma of esophagus → Histopathology → 1. Squamous cell carcinoma · **Book p. 71** (1st half PDF p. 74)

What esophageal SCC looks like under the microscope

It is made of **islands of neoplastic squamous cells**. Differentiated tumours also show **epithelial cell nests**.

PATHOLOGY

GIT Others p. 14
C4-1
ledger TU-06



Fits after: Carcinoma of esophagus → effects and complications · **Book p. 71** (1st half PDF p. 74)

A malignant ulcer can bleed fatally

As well as dysphagia and haematemesis, **perforation of the malignant ulcer can cause fatal haemorrhage.**

PATHOLOGY

GIT Others p. 14
C4-2
ledger TU-07

Fits after: Gastric adenocarcinoma → Predisposing factors / intestinal type “on top of intestinal metaplasia” · **Book p. 72** (1st half PDF p. 75)

H. pylori → cancer: the last steps

The sequence continues past intestinal metaplasia: H. pylori gastritis → atrophic gastritis → intestinal metaplasia → **dysplasia → cancer.**

GIT Others lists intestinal metaplasia as a **precancerous (dysplastic) lesion** of the stomach.

PATHOLOGY

GIT Others p. 15
C4-3
ledger TU-12

Fits after: Gastric adenocarcinoma → “It may take one of two types” · **Book p. 72** (1st half PDF p. 75)

Name and frequency of the two types

The intestinal and diffuse types make up the **Lauren classification:** intestinal type **53%**, diffuse type **33%**, unclassified **14%**.

PATHOLOGY

GIT Others p. 16
C4-4
ledger TU-16

Fits after: Late gastric cancer → (2) Malignant ulcer · **Book p. 72** (1st half PDF p. 75)

The malignant gastric ulcer

The ulcer is typically **more than 5 cm** across, with an everted edge, a **necrotic floor** and a **firm base.**

PATHOLOGY

GIT Others p. 16
C4-5
ledger TU-15

Fits after: Spread of gastric carcinoma → Blood spread · **Book p. 73** (1st half PDF p. 76)

Blood spread goes beyond the liver

Blood-borne metastases go to the liver and also to the **lung, bone and brain.**

PATHOLOGY

GIT Others p. 16
C4-6
ledger TU-19

Fits after: Neoplastic adenomatous polyp of the colon · **Book p. 73** (1st half PDF p. 76)

Colonic adenomas run in families

There is a **well-defined familial predisposition** to colonic adenomas. They predispose to colorectal carcinoma.

PATHOLOGY

GIT Others p. 17
C4-7
ledger TU-23

Fits after: Tubular adenoma / Villous adenoma · **Book p. 73** (1st half PDF p. 76)

Tubular vs villous adenoma: size and cancer risk

	Tubular adenoma	Villous adenoma
Size	Small, <1 cm	Large, >2 cm
Number	Multiple	Solitary
Dysplasia / cancer	Rare	High (about 40%)

GIT Others’ table gives the 40% for adenomas >4 cm; its text gives it for villous adenomas in general.

PATHOLOGY

GIT Others p. 18
C4-8
ledger TU-25





07

Foodborn Infection

Microbiology · Internal Medicine · Main book pp. 106–114 **1st-half file**

First-half file, PDF pp. 109–117. GIT Others pp. 22–33 repeat this chapter almost word for word, so only small laboratory and control details are new.

Fits after: Salmonella → Culture and Biochemical reactions · **Book p. 107** (1st half PDF p. 110)

Salmonella culture and sugar fermentation

- ◆ A **facultative anaerobe** that grows well at **37 °C in 24 hours**. Colonies on MacConkey are **pale (yellow)** because it does not ferment lactose.
- ◆ Most species ferment sugars with **acid and gas**. **S. typhi makes acid only** (no gas).

MICROBIOLOGY

GIT Others p. 22
C7-1
ledger FI-03, FI-05

Fits after: Enteric fever → Laboratory diagnosis → A. Isolation (first week) · **Book p. 109** (1st half PDF p. 112)

How the first-week blood culture is done

Blood is cultured in **fluid (broth) media**, then **subcultured on solid media**, mainly MacConkey, at 37 °C.

MICROBIOLOGY

GIT Others p. 24
C7-2
ledger FI-10

Fits after: Enteric fever → Prevention and Control (before "1 - Immunization") · **Book p. 109** (1st half PDF p. 112)

Control starts with food handlers

Food handlers are examined to detect carriers. Carriers are treated or kept away from handling food. GIT Others lists this before immunization.

MICROBIOLOGY

GIT Others p. 25
C7-3
ledger FI-16

Fits after: Salmonella food poisoning → A. S. typhimurium · **Book p. 110** (1st half PDF p. 113)

S. typhimurium food poisoning is self-limiting

After lasting 2–5 days, **it usually resolves on its own (spontaneous cure)**.

MICROBIOLOGY

GIT Others p. 25
C7-4
ledger FI-18

Fits after: Campylobacter → Laboratory diagnosis · **Book p. 111** (1st half PDF p. 114)

Campylobacter laboratory details

- ◆ A **rectal swab** is an acceptable specimen, collected with care to avoid oxygen exposure.
- ◆ Darting motility is seen in a wet mount under **dark-field illumination**.
- ◆ Isolated colonies are identified by **biotyping, serotyping and phage typing**.

MICROBIOLOGY

GIT Others p. 27
C7-5
ledger FI-23

Fits after: Brucella → Culture · **Book p. 111** (1st half PDF p. 114)

Brucella needs CO₂

Brucella is aerobic, but culture needs **10% CO₂** as well as serum-enriched media.

MICROBIOLOGY

GIT Others p. 27
C7-6
ledger FI-26





Fits after: Enteric fever → Interpretation of Widal test · **Book p. 109** (1st half PDF p. 112)

Widal test: what a high H titre means

- ◆ Your book p. 109: a high titre to **one** H antigen ($\geq 1:160$) suggests recent infection with that type; high O titre **plus more than one** H antigen indicates recent immunization.
- ◆ GIT Others p. 24: “**More than one** H antigen ($\geq 1:160$) suggest recent infection with this type of Salmonella **and** recent immunization.”

The GIT Others list here is garbled (its items 1–2 are cut off), so it is not a reliable correction. Ask your teacher before changing what you learned.

MICROBIOLOGY

CHECK

GIT Others p. 24
C7-D1
ledger FI-13

14

Polyposis of Colon

Surgery · Pathology · Main book pp. 189–194 **1st-half file**

First-half file, PDF pp. 192–197.

Fits after: Familial polyposis coli → “These are adenomatous polyps” · **Book p. 190** (1st half PDF p. 193)

Polyp type in FAP

In FAP, **most of the polyps are tubular adenomas.**

PATHOLOGY

GIT Others p. 19
C14-1
ledger TU-29

THE TWO BOOK FILES JOIN INSIDE CHAPTER 15 (PRINTED P. 197)

15

Colorectal Cancer

Surgery · Pathology · Main book pp. 195–209 **Spans both files**

Pages 195–197 are in the first-half file (PDF pp. 198–200); pages 197–209 are in the second-half file (PDF pp. 1–13).
Page 197 is in both files.

Fits after: 3 - Microscopic picture → Adenocarcinoma · **Book p. 196** (1st half PDF p. 199)

Adenocarcinoma: 98% of large-bowel cancers

Your book says “more than 90%” of colorectal carcinomas are adenocarcinomas. GIT Others gives a precise figure: **98%** of all cancers of the large intestine are adenocarcinomas.

PATHOLOGY

GIT Others p. 19
C15-1
ledger TU-30

Fits after: 3 - Microscopic picture → Mucinous adenocarcinoma (variant) · **Book p. 196** (1st half PDF p. 199)

Two kinds of mucoid (mucinous) carcinoma

- ◆ **Mucinous adenocarcinoma:** abundant *extracellular* mucin (your book: >50% mucinous material).
- ◆ **Signet ring cell carcinoma:** the mucin stays *inside* the malignant cell and pushes the nucleus to one side (eccentric nucleus).

PATHOLOGY

GIT Others p. 20
C15-2
ledger TU-36

Fits after: Aetiology → 1 - Predisposing factors · **Book p. 196** (1st half PDF p. 199)

Add irradiation to the predisposing factors

GIT Others lists **irradiation** as a predisposing factor for colorectal adenocarcinoma, along with adenoma, familial polyposis, IBD and gene mutations. It is not in your book’s list.

PATHOLOGY

GIT Others p. 19
C15-3
ledger TU-32



Fits after: Pathology of colorectal carcinoma → Age · **Book p. 196** (1st half PDF p. 199) · also Ch 4, p. 74 (1st-half PDF p. 77)

Age at presentation: 50–70 or 60–79 years?

- ◆ GIT Others p. 19: “Age of **50–70** years.”
 - ◆ GIT Others p. 20 and your book Ch 4 p. 74: “between **60 and 79** years of age”.
 - ◆ Your book Ch 15 p. 196: “old age, however no age is immune”.
- GIT Others contradicts itself. Ask your teacher which range to learn.

PATHOLOGY

CHECK

GIT Others p. 19, 20
C15-D1
ledger TU-31

25

Pyogenic Liver Abscess

Surgery · Internal Medicine · Parasitology · Main book pp. 324–331 **2nd-half file**

Second-half file, PDF pp. 128–135. The amoebic liver abscess section is on pp. 329–331.

Fits after: Amebic liver abscess → Investigations → Lab (CBC) · **Book p. 331** (2nd half PDF p. 135) · also Ch 9, p. 138 (1st-half PDF p. 141)

Blood count in amoebic liver abscess

- ◆ Your book Ch 25 p. 331: “CBC: Leukocytosis and **eosinophilia**”.
- ◆ GIT Others p. 47, and your book Ch 9 p. 138: CBC shows **polymorphonuclear leucocytosis**, which helps separate amoebic liver abscess from viral hepatitis. Neither mentions eosinophilia for amoebic liver abscess.

Both agree on leucocytosis. Check with your teacher whether eosinophilia should be learned.

PARASITOLOGY

CHECK

GIT Others p. 47
C25-D1
ledger PA-29



The other 34 chapters: checked, nothing to add

Every topic in GIT Others was matched to your book's chapters in both files. In these chapters GIT Others covers the topic, but your book already says it:

Ch	Chapter	Book pages	Subject	Where your book already has it
09	Parasitic GI Diseases	pp. 122–139	PARASITOLOGY	GIT Others pp. 35–47 (Schistosoma, Taenia, Ascaris, Enterobius, hepatic amoebiasis) are already here word for word, with the same doses and time periods.
19	Acute Hepatitis	pp. 251–263	MICROBIOLOGY	Leptospirosis and yellow fever liver involvement are already in Ch 7 (pp. 113–114).
22	Liver Cirrhosis	pp. 280–307	PARASITOLOGY	Hepatosplenic schistosomiasis is already on p. 283 and in Ch 9.
23	Portal Hypertension	pp. 308–317	PARASITOLOGY	Schistosomal periportal fibrosis (p. 310) and PPIs for bleeding varices (Ch 3 p. 47) are already covered.
24	Hydatid Cyst	pp. 318–323	PARASITOLOGY	GIT Others names hydatid disease only as a differential of amoebic liver abscess; Ch 9 already does.
35	Obstructive Jaundice	pp. 389–398	PARASITOLOGY	Biliary Ascaris is already on p. 390; the other Ascaris details are all in Ch 9.

No matching material in GIT Others: chapters 1, 2, 5, 6, 8, 10, 11, 12, 13, 16, 17, 18, 20, 21, 26, 27, 28, 29, 30, 31, 32, 33, 34, 36, 37, 38, 39, 40.

Points to check

In these places your book and GIT Others disagree, or your book disagrees with itself. Each entry quotes both versions and chooses neither. Tap an ID to open the entry.

Entry	Ch	Subject	Topic	Main book	Source
C3-D1	3	PHARMACOLOGY	Misoprostol: PGE1 or PGE2 analogue?	Book p. 37 (1st half PDF p. 40) also p. 48 (1st-half PDF p. 51)	GIT Others p. 4
C3-D2	3	PHARMACOLOGY	Levofloxacin dose in the rescue regimen	Book p. 41 (1st half PDF p. 44) also p. 48 (1st-half PDF p. 51)	GIT Others p. 5
C3-D3	3	PATHOLOGY	Commonest site of gastric cancer: how common?	Book p. 62 (1st half PDF p. 65)	GIT Others p. 15
C7-D1	7	MICROBIOLOGY	Widal test: what a high H titre means	Book p. 109 (1st half PDF p. 112)	GIT Others p. 24
C15-D1	15	PATHOLOGY	Age at presentation: 50–70 or 60–79 years?	Book p. 196 (1st half PDF p. 199) also Ch 4, p. 74 (1st-half PDF p. 77)	GIT Others p. 19, 20
C25-D1	25	PARASITOLOGY	Blood count in amoebic liver abscess	Book p. 331 (2nd half PDF p. 135) also Ch 9, p. 138 (1st-half PDF p. 141)	GIT Others p. 47

رَبِّ زِدْنِي عِلْمًا

[سورة طه: 114]



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Pediatric 1



Pediatrics 2



Surgery



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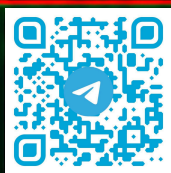
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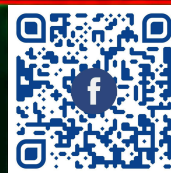
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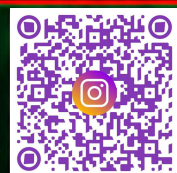
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